



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
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May 14, 2026

VIA ELECTRONIC MAIL

Evan Escobar
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with Valley Children's Hospital
Block Transfer Filing No. 2026-HOSP-293
Filed on March 17, 2026

Dear Mr. Evan Escobar:

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **Valley Children's Hospital** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **July 1, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. On the Block Transfer Filing form, in the Filing Info section, the Plan indicated there were zero (0) bed days utilized and zero (0) admits at the Terminating Hospital for the most recently completed Calendar Year. However, in the Enrollee Access section of the Block Transfer Filing form, Plan indicated that **LaSalle Medical Associates** has 91,823 assigned enrollees, utilized 79 bed days, and 35 non-ER admits. Please confirm the following:
 - a. The total number of affected enrollees,
 - b. The total number of bed days used,
 - c. The number of non-ER admits,

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- d. Whether LaSalle Medical Associates has routine access to the Terminating Hospital,
 - e. If the pending contract termination impacts 50,000 or more enrollees, please submit the relevant portions of the Provider Contract(s) that relate to continuity of care and transition of care.
2. For each affected provider group that utilizes the Terminating Hospital, please summarize or confirm the following:
 - a. Whether the provider group has been contacted,
 - b. Name of contact, phone number, and email address,
 - c. Whether the provider group expressed concerns,
 - d. Whether the provider group has routine access to the Terminating Hospital,
 - e. The ability of the provider group to redirect enrollees to the Alternate Hospitals:
 - i. Whether there is any specialty for which all of the associated specialists have admitting privileges **only** at the Terminating Hospital and not at any of the Alternate Hospitals,
 1. If yes, please identify the specialty and explain how Plan enrollees assigned to the provider group will receive care and services for that specialty following contract termination.
 - ii. If the provider group does not have any primary care physicians or specialists with admitting privileges to one or more Alternate Hospitals, please explain how its assigned enrollees will be admitted to the Alternate Hospital(s) (e.g. via hospitalists, Plan admission process, etc.).
 1. If hospitalists are utilized, please indicate:
 - a. The name of the group,
 - b. The number of associated hospitalists, and
 - c. If the hospitalists can admit enrollees for any medically necessary services.
3. In the event of contract termination where the Plan removes the Terminating Hospital from its contracted provider network, please summarize or confirm the following that may result:
 - a. The total number of enrollees, if any, who have a residence or workplace beyond 30 minutes or 15 miles of a contracting or plan-operated hospital,
 - b. The number of enrollees from 3.a. within each corresponding zip code,
 - c. The Plan's process for ensuring compliance with Cal. Code Regs., Tit. 28, § 1300.51(d)(H)(ii). (ii) Hospitals. In the case of a full-service plan, all enrollees have a residence or workplace within 30 minutes or 15 miles of a contracting or plan-operated hospital which has a capacity to serve the entire dependent enrollee population based on normal utilization, and, if

separate from such hospital, a contracting or plan-operated provider of all emergency health care services.

4. For provider group, La Salle Medical Associates, the Alternate Hospital detailed in the Services Comparison of this Filing do not provide Acute Neonatal, Acute Pediatric, and Hemodialysis services. Please summarize how the Plan will ensure enrollees will maintain access to Acute Neonatal, Acute Pediatric, and Hemodialysis services at an in-network benefit level post-termination.
5. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.

Please submit the Plan's response by **May 20, 2026**, via the Block Transfer portal as a "Response to Additional Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,

A handwritten signature in black ink, appearing to read "Cindy Nguyen". The signature is fluid and cursive, with the first name "Cindy" being more prominent than the last name "Nguyen".

Cindy Nguyen
Analyst II
Office of Plan Monitoring